When Clients Say, “I’ve Got No Motivation”

Leyton Gray

October 2026

Most staff working in substance use services will have heard clients say, “I’ve just got no motivation.” It’s a familiar phrase, but one that deserves a little more thought.

Difficulty initiating or sustaining change can be influenced by a wide range of factors. Some are well recognised, others less so. Many coexist and interact, making it unlikely that any single factor will fully explain what is happening. This article explores some of those factors and how they might present in practice. It isn’t intended to be an assessment tool or a checklist, but a framework for thinking about low motivation. These aren’t boxes to work through. They’re possibilities to consider in the context of the person in front of us. Not every section will apply to every client, nor is the list intended to be exhaustive.

Understanding motivation

Motivation is often spoken about as though it were a single thing; something a person either has or doesn’t have. In practice, it is considerably more complex. When someone says they have no motivation, they may be describing fatigue, emotional flatness, difficulty getting started, an inability to experience much reward from everyday activities, a lack of confidence that their efforts will achieve anything, or simply having no clear idea what they are working towards. Two people can therefore present in much the same way while having very different things going on underneath.

Difficulty initiating or sustaining change is not, in itself, evidence that someone doesn’t care or isn’t trying. Before reaching that conclusion, it is worth considering the range of biological, psychological and social factors that may be influencing their ability to act.

People can also want more than one thing at the same time. Continuing to use substances may still provide pleasure, belonging, relief, identity, routine or escape. Reducing or stopping may matter to them as well. Both can be true. This is sometimes described as ambivalence. Whatever terminology we use, it is a normal part of considering change. Someone may understand the consequences of continuing and have good reasons for wanting something different, while remaining unsure whether change is worth what they would have to give up. Sometimes what looks like low motivation is this conflict playing itself out.

It is also worth asking whose goal we are using to judge motivation. Someone may appear poorly motivated because we are measuring their motivation against an outcome that matters more to us, their family or the service than it currently does to them. Their behaviour may make considerably more sense when viewed in relation to what matters to them. Looking at where they already demonstrate initiative, persistence or commitment can sometimes tell us as much as looking at where they appear to struggle.

Physical factors

Long-term opioid use can affect testosterone levels. For some people this may contribute to fatigue, reduced libido, reduced strength and muscle mass, poorer physical recovery and a more general loss of drive. For substance use workers, the important point is awareness. A client is unlikely to present saying that their testosterone is low. They may talk about being tired all the time, losing interest in sex, feeling physically weaker or simply not feeling like themselves.

These symptoms are non-specific and can have many causes. Our role is not to diagnose or investigate them. Where there is a persistent or significant change, particularly where several symptoms occur together, it may be appropriate to discuss this with the prescribing team or encourage the client to raise it with their GP. Some clients may already have been told that low testosterone is simply a consequence of taking methadone. Methadone may indeed be contributing. Knowing the likely cause, however, doesn’t tell us how much it is affecting the person or whether anything further needs to be considered.

Sleep deserves attention too. A client may report sleeping eight or ten hours and still wake feeling exhausted. Time spent asleep and restorative sleep are not necessarily the same thing. Opioids can affect sleep quality and are associated with sleep-disordered breathing in some people. Alcohol, stimulants, benzodiazepines and other medication can further complicate the picture. Pain, anxiety, irregular routines and the circumstances in which someone is living may be just as important. The consequences are familiar: fatigue, poorer concentration, irritability, reduced tolerance for frustration and difficulty initiating activity. If somebody is chronically exhausted, trying harder is unlikely to be the complete answer.

Substance use populations also carry a significant burden of physical ill health. Chronic pain, respiratory disease, anaemia, endocrine problems, infection and a range of other conditions can all present partly through tiredness, reduced activity and what gets described as poor motivation. Pain is particularly important. Living with persistent pain consumes attention and energy. It interferes with sleep, reduces activity and can gradually shrink the range of things someone does. Other prescribed medication may also contribute. Sedation, cognitive slowing and fatigue may come from several sources, particularly where people are taking multiple medications. A change in motivation can therefore sometimes warrant a fairly ordinary question: How are they physically?

Reward, effort and getting started

Some clients describe neither depression nor unhappiness. They describe feeling flat. Things are “fine”, but very little feels particularly exciting, interesting or worth pursuing. Much of everyday motivation comes from anticipation. We do things partly because we expect something rewarding at the other end: enjoyment, satisfaction, connection, achievement or simply the feeling that we have accomplished something. When that anticipation becomes muted, the pull towards activity becomes weaker. Someone may understand perfectly well that going to the gym, meeting a friend or completing a course would be good for them. Knowing that intellectually and feeling drawn towards doing it are different things.

Someone may want the outcome very much while having little enthusiasm for the activities required to achieve it, particularly when the rewards are distant and the effort or discomfort is immediate. The amount of effort involved matters too. Imagine that the value of an outcome remains the same, but the effort involved in achieving it feels twice as great. The calculation changes. This can help explain a familiar presentation: somebody genuinely wants something, talks convincingly about why it matters and then repeatedly fails to take the first step. In those circumstances, increasing encouragement may achieve relatively little. Reducing the amount of effort required to begin may achieve considerably more.

It is also worth remembering that motivation often follows action. People can spend a long time waiting to feel ready, interested or confident enough to begin. Yet getting started can itself change how they feel about continuing. Action creates opportunities for interest, confidence and a sense of progress to develop, sometimes providing the motivation that was missing beforehand.

The first step sometimes needs to be very small. It also helps if the person can see what that step is towards. Values can be useful here, but every action doesn’t need to become a lengthy exploration of what matters. Sometimes simply recognising that a small action moves them towards something they care about is enough. A ten-minute walk may seem insignificant on its own. If it is a step towards getting fitter, leaving the house more often, seeing their children, returning to work, or simply feeling more like themselves, it has a different context. The aim is to make action achievable while keeping some connection to why it is worth taking.

There are also times when the difficulty is less about the effort involved and more about what the action brings with it. Making a phone call, attending an appointment, opening a letter, going to a group or applying for something may bring anxiety, shame, uncertainty, anticipated judgement or the possibility of another disappointment. Avoidance can work very well in the short term. The difficult feeling reduces when the thing is put off, which makes avoiding it again more likely. From the outside, this can look like procrastination, disengagement or simply not being motivated.

Making the task smaller may help, but sometimes the difficulty has to come along too. The useful question then becomes what would help the person take the step while the discomfort is present, particularly where they can see that step as moving towards something that matters to them.

Executive function may also be relevant. Some people have always struggled with getting started, organising themselves, estimating time, prioritising competing demands or following tasks through to completion. These difficulties may pre-date substance use by many years. ADHD is particularly relevant within substance use services, although executive function difficulties are not confined to people with an ADHD diagnosis. Neurodevelopmental differences, acquired brain injury, trauma, poor sleep, chronic stress and substance use itself may all affect somebody's ability to organise and execute what they intend to do.

The result can be a significant gap between what somebody intends to do and what they manage to put into practice. A person may genuinely intend to attend an appointment, complete paperwork or begin a course and still repeatedly fail to do so. Practical support can sometimes achieve more than another conversation about motivation: reminders, prompts, simplifying tasks, reducing the number of steps involved and establishing routines.

Stimulants and the reward system

Previous and current stimulant use deserves particular consideration. Drugs such as crack cocaine and methamphetamine produce very large and rapid changes in dopamine signalling. Repeated exposure to intense reward can alter how everyday activities are experienced afterwards. Everyday life cannot compete with crack cocaine for intensity.

Work, hobbies, exercise, cooking, relationships and small achievements operate on an entirely different scale. During recovery, clients may find these activities remarkably unrewarding even when they remember previously enjoying them. This can easily be described as having "no motivation". After repeated exposure to very intense rewards, everyday activities can take time to feel satisfying again. Recovery is gradual, and repeated experiences of ordinary reward have a part to play.

There is another element worth considering: chasing intensity. For some people, the attraction to intensity itself becomes familiar. Life can feel slow. Progress can feel painfully incremental. For some people, experiences that provide immediate and intense rewards may remain particularly appealing, while slower, less immediately rewarding activities can be difficult to engage with. Part of recovery can therefore involve learning to tolerate and appreciate less intense experiences again.

A cup of coffee with somebody, finishing a job, walking somewhere, cooking a decent meal or noticing that something has improved since last month will never reproduce the intensity of a stimulant high. They aren't supposed to. Over time, however, these

experiences can begin to matter again.

Purpose, identity and expectations

There is another question that sits beyond brain chemistry and physical health: What am I getting motivated for?

For some clients, substance use has occupied a considerable amount of time, attention and effort. Obtaining substances, managing withdrawal, finding money or dealing with associated difficulties may have become familiar parts of their routine. People can develop considerable resourcefulness and persistence in managing these demands. As substance use reduces or life becomes more stable, the routines and priorities that previously organised much of someone's time may begin to change. This can create space for other interests and activities, but it may also leave gaps that are difficult to fill. For some people, the absence of familiar demands can initially feel surprisingly empty.

There is also identity. Substance use may have occupied a significant part of somebody's adult life. Their relationships, routines, social group and sense of themselves may all have developed around it. Stopping or reducing substance use removes something. It does not automatically provide the replacement. Finding out what matters now can take time.

Loss and grief may also be relevant. Some people have experienced the loss of relationships, health, employment, housing, contact with children or people close to them. Other losses may become more apparent as circumstances change. People may begin to recognise years that have gone, opportunities that will not return or versions of themselves they thought they would become. Grief does not always present as visible sadness. It can look like withdrawal, inertia or disengagement.

Shame can have a similar effect. Someone who has repeatedly experienced failure, rejection or disappointment may become increasingly reluctant to place themselves in situations where those experiences could happen again. Sometimes doing nothing feels safer than trying and failing.

Wanting something and believing you can achieve it are different. Some clients have made repeated efforts to change aspects of their lives without achieving the outcomes they hoped for. They may have engaged with treatment, reduced or stopped substance use for periods, pursued employment or worked to improve relationships, only to encounter further difficulties or setbacks. Eventually, expectations change. If previous experience repeatedly teaches somebody that effort ends in disappointment, it could become harder to invest in the next attempt.

Previous experiences of services may also shape expectations. Someone who has repeatedly attended assessments, discussed goals, agreed plans or been referred elsewhere without experiencing much meaningful change may understandably be reluctant to invest in another conversation about what they intend to do differently. Their

apparent lack of motivation may tell us something about their expectations of the service as well as their expectations of themselves.

This is one reason achievable goals matter. Small successes provide evidence that effort can produce an outcome, particularly important when somebody has spent years accumulating evidence to the contrary.

Environment

Housing instability, debt, poverty, loneliness, unsafe relationships, unemployment and limited access to meaningful activity all affect what somebody is realistically able to do. Chronic stress consumes mental bandwidth. Some clients are being asked to make significant behavioural changes while managing considerable pressures on their time, energy and resources.

Environment also shapes what rewards are available. Stopping drug use may mean losing a social network as well as a substance. If nothing replaces that connection, recovery can become remarkably lonely. Telling somebody to develop hobbies is relatively easy. Building a life containing people, places and activities that genuinely matter to them is considerably harder.

Bringing it together

These factors rarely occur neatly in isolation. Poor sleep may affect energy and executive function. Pain can interfere with sleep and activity. Stimulant use may affect how rewarding everyday activities feel, while social isolation limits opportunities to experience those rewards. Shame, chronic stress, financial pressure and uncertainty about the future may complicate things further.

This is where formulation becomes useful. When somebody tells us they have no motivation, it is worth getting curious about what they mean and what may be making action difficult. Do they want to do things but struggle to get started? Does very little feel rewarding? Are they exhausted, overwhelmed or expecting another disappointment? Are the goals genuinely theirs? Sometimes one issue will stand out; often several will need attention.

Different difficulties suggest different responses. Physical symptoms may warrant discussion with the appropriate clinician. Executive difficulties may respond to structure, reminders and simpler tasks. Someone whose confidence has repeatedly been damaged may benefit from achievable experiences of success. Avoidance may call for support in taking action while discomfort is present. Where life has become empty, increasing activity without considering meaning may simply produce a busier empty life.

Sometimes the useful intervention is helping somebody understand what is happening. There is a considerable difference between believing, “I’m lazy” and recognising, “I’m

struggling to get started, and there are reasons for that which we can work on."

There is also a risk in giving people explanations. Something intended to increase understanding can become another reason to believe change is impossible. A client could reasonably read about poor sleep, low testosterone, executive difficulties, stimulant-related changes, chronic stress, shame and loss and conclude that they have everything stacked against them. Each factor we recognise, however, gives us another possible place to work.

In summary

- "No motivation" describes an experience. It doesn't tell us what is producing it.
- Similar behaviour can arise for very different reasons, with several factors operating at the same time.
- People can have genuine reasons both for changing and for continuing as they are.
- What looks like poor motivation may sometimes reflect the goal we are measuring it against.
- Looking at where somebody already demonstrates initiative or persistence can help us understand where they struggle.
- Understanding what makes action difficult can help us identify a place to begin. Motivation often develops through action itself.
- Explanations should open up possibilities for action, rather than become another list of reasons why change cannot happen.

This framework is intended to inform conversations rather than provide an answer. Understanding why motivation is difficult for somebody is often the beginning of the conversation rather than the end.